Suboccipital Decompressive Craniectomy for Cerebellar Infarction

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Study Design: Retrospective Cohort

Abstract

Background: Space-occupying cerebellar infarction can lead to brainstem compression and obstructive hydrocephalus.

Methods: We reviewed 42 patients with space-occupying cerebellar infarction. Twenty-one patients underwent suboccipital decompressive craniectomy (SDC) within 24 hours, and 21 patients received conservative medical management.

Total sample size: 42 patients
Surgical group: 21 patients
Control group: 21 patients

Results

The 30-day mortality rate was significantly lower in the surgical group (23.8%) compared to the control group (71.4%).

Favorable outcome (mRS 0-3) was achieved in 57.1% of surgical patients vs 14.3% of control patients.

Surgical type: SDC alone
Timing: <24h from symptom onset

Inclusion Criteria:
- Cerebellar infarction with mass effect on CT/MRI
- Age 18-80 years
- Admission within 48 hours of symptom onset
- Glasgow Coma Scale score ≥8